



RAFA PRIMARY ASSESSMENT AND MEDICAL NOTES

SCENE SAFETY AND OVERALL IMPRESSION	Decide to help. <i>At a minimum, call dispatch and 911 (EMS).</i>	PPE/BSI <i>Put your gloves and glasses on.</i>	Scene Safety <i>Is my Scene Safe? Only help if it is safe.</i>	# of Injured or Ill <i>How many people will I be helping?</i>
	Determine: • Mechanism of Injury (MOI) for Trauma • Nature of Illness (NOI) for Medical	Spinal Stabilization <i>Does the MOI indicate I should stabilize the neck or spine?</i>	Moving an Injured or Ill Person <i>Can I help the person where they are at? Will I have to move the person to safety?</i>	Transport <i>Choose a transport plan. Either wait for EMS, rendezvous with EMS, or request air transport. This may change.</i>
PRIMARY ASSESSMENT VITALS A.B.C.D.E.	General Impressions <i>Do they look normal? Is something not right? What could have happened to them?</i>	Obtain Consent <i>Do you want help? Is it ok if I try to figure out what is going on? Is it ok to touch you?</i>	Chief Complaint <i>What does the person or the first aid provider think is wrong?</i>	Airway <i>Does the person have an open airway? If unresponsive, open the airway.</i>
	Breathing <i>Are they breathing? Are they having difficulty breathing? (RR 12-20)</i> Not Breathing <i>Start CPR.</i>	Circulation <i>Do they have a pulse? Is heavy bleeding controlled? (HR 60-100 bpm) Skin: pink, warm, dry)</i>	Disability <i>Check the head, neck, and spine for injuries based on MOI. (Pupils: equal and reactive to light.)</i>	Environment <i>Address conditions creating immediate threats such as extreme weather, heat, or cold.</i>
LEVEL OF RESPONSIVENESS (LOR) A.V.P.U.	Alert <i>Alert to environment and speaks normally.</i>	Verbal Stimulus <i>Responds to commands.</i>	Painful Stimulus <i>Responds to pain. This is an emergency.</i>	Unresponsive <i>Does not respond. This is an emergency.</i>
	Questions <i>Can you tell me your name? Can you tell me what happened? Do you know where you are? What day is it?</i>			A positive response is considered Alert and Oriented x4.
FINDING OUT THE PROBLEM S.A.M.P.L.E	Signs and Symptoms <i>What's going on today? What hurts?</i>	Allergies <i>Are you allergic to any medication or other things?</i>	Medications <i>Are you on any medications?</i>	Previous Medical History <i>Any health or medical issues?</i>
	Last Oral Intake and Output <i>When was the last time you had anything to eat or drink?</i>		Events Leading Up to Illness/Injury <i>What were you doing before this happened? Has anything like this happened before?</i>	
CRITICAL MEDICAL PROBLEMS CONSIDER RAPID TRANSPORT TO MEDICAL CARE	Headache <i>Is this different from other headaches? Are you nauseous?</i> Action: Stroke test, seek medical attention.	Chest Pain <i>Does the pain radiate down your arms? Are you short of breath?</i> Action: Prepare for CPR, rapid transport.	Stomach Ache <i>Is the pain severe? Is it caused from trauma? Is there pain to palpation?</i> Action: Seek medical attention. If severe, rapid transport.	Allergy <i>Are you allergic to anything? Are your lips, tongue, or throat swelling?</i> Action: EpiPen or Benedryl. If severe, rapid transport.
	Stroke - FAST Face - Droop Arms - Weakness Speech - Difficulty Time - Critical Action: Rapid transport.	Overdose <i>Suspected opioid overdose.</i> Action: Administer Naloxone or Narcan. Rapid transport.	Diabetic Emergency <i>Have you experienced low blood sugar before? Do you have diabetes?</i> Hypoglycemia: Low blood sugar Action: Provide juice or carbohydrates if the person is alert. Never give Insulin in the field. Seek medical attention if they don't improve.	



RAFA SECONDARY ASSESSMENT AND TRAUMA NOTES

SECONDARY ASSESSMENT HEAD TO TOE EXAM	Head and Face <i>Look behind the ears. Check eyes with a penlight, pupils should be equal and reactive.</i>	Neck <i>Check for pain to palpation and look for irregularities.</i>	Chest <i>Squeeze sides, push down at the sternum. Look for even chest rise.</i>	Abdomen <i>Palpate all four quadrants, note if there is pain on pressure or rebound.</i>
	Pelvis <i>Check for pain and stability.</i>	Extremities <i>Check for DOTS.</i>	Back <i>Log roll and feel for DOTS or blood on back.</i>	Monitor Vitals <i>Check LOR, HR, RR, pupils, and skin signs.</i>
INJURIES D.O.T.S	Deformities <i>(Misalignment, Bruising)</i>	Open Wounds <i>(Bleeding)</i>	Tenderness <i>(Pain to touch)</i>	Swelling <i>(Enlarged)</i>
BLEEDING CONTROL STOP THE BLEED	Direct Pressure <i>Have the injured person or provider apply pressure on the wound.</i> Pressure Bandage <i>Use for wounds on the chest, abdomen, or other large surfaces.</i>	Tourniquets <i>Apply "high and tight" or at least several inches above the wound.</i> <i>Note: Injured person will feel PAIN.</i>	Wound Packing <i>Pack junctional or large wounds. Find the bleeding artery or vein. Fill wound cavity with gauze and bandage while maintaining pressure.</i>	Wound Care <i>Irrigate wound to remove debris. Clean and apply antibiotic cream. Use sterile dressings. Keep bandages clean.</i>
EVALUATING PAIN O.P.Q.R.S.T.	Onset <i>When did it start?</i>	Provocation <i>Does anything make it better or worse?</i>	Quality <i>Is it sharp, dull, piercing? Can you describe the pain?</i>	
	Radiation <i>Does the pain travel anywhere else?</i>	Severity <i>On a 1 to 10 scale, how bad is the pain?</i>	Time <i>How long have you been having this problem?</i>	
CARE FOR JOINT INJURIES R.I.C.E.	Rest <i>Have the person take pressure off the injured area and rest.</i>	Immobilize <i>Leave in a position of comfort with the injured person holding the area or splint the affected extremity.</i>	Cold <i>Add an ice pack or bag of ice. Do not put ice directly on skin; use a thin cloth or a triangular bandage.</i>	Elevate <i>Elevate the injured area, if possible, or if it does not cause the injured person additional pain.</i>
SPLINTING P.M.S	Splinting <i>Use rigid materials to splint in a position of comfort, immobilize joints above and below injured area.</i>	Pulse <i>Check for a pulse past the injured area before and after splinting.</i>	Motor Function <i>Have injured person make small movements before and after splinting.</i>	Sensation <i>Ask injured person if they can feel where you are touching them.</i>
DOCUMENT INJURIES S.O.A.P	Subjective <i>What the person tells you and Chief Complaint.</i>	Objective <i>What you find during assessment.</i>	Assessment <i>What you think is happening.</i>	Plan <i>The plan for treatment and transport.</i>
BURNS RULE OF NINES	AREA	APPROXIMATE SURFACE	FRONT	BACK
Superficial (1st degree) - Red, painful, swollen. Partial Thickness (2nd degree) - Red, painful, swollen, and blistered. Full thickness (3rd degree) - Pale, scalded, and charred. Treatment - Cool, keep clean and dry. Separate burned fingers and toes. Maintain body temperature.		Head 9%		
		Chest 18%		
		Back 18%		
		Arms 9%		
		Legs 18%		
		Genitals 1%		